PROPOSAL FOR ASLIN LANGUAGE PARTNERSHIP AND CLINICAL EVALUATION

SUBMITTED BY	Unmute
PROGRAMME	Unmute Healthcare Communication Initiative
PROPOSED PARTNER	Association of Sign Language Interpreters of Nigeria (ASLIN)
PURPOSE	Language Partnership, Interpreter Review, and Clinical Evaluation

1 EXECUTIVE SUMMARY

Unmute is an AI-powered communication technology initiative focused on reducing communication barriers between Deaf individuals and hearing people.

Our initial product focuses on healthcare, where communication barriers between Deaf patients and healthcare professionals can make routine interactions difficult, time-consuming, and stressful.

Deaf patients may struggle to communicate symptoms, describe discomfort, understand medical instructions, or respond to questions during consultations. Healthcare professionals may also lack the sign-language skills or immediate access to qualified interpreters needed to facilitate these interactions.

Unmute is developing a technology-assisted communication platform designed to support two-way communication between Deaf patients and hearing healthcare professionals through sign-language recognition and speech-to-sign technology.

A language is not a dataset to be collected. It belongs to the people who use it, and in Nigeria it is carried by interpreters and by Deaf communities whose signs differ from the American Sign Language that most recognition systems are trained on. A tool built without them will be confident and wrong.

Unmute is therefore approaching ASLIN before approaching hospitals. We would like to review our initial clinical phrase list with interpreters who work in Nigerian hospitals, and to understand where ASL would mislead a Nigerian Deaf patient rather than help them.

2 THE PROBLEM

Communication accessibility remains a significant challenge for Deaf patients accessing healthcare services.

During routine hospital interactions, communication may depend on:

- Written notes or text messages.
- Lip-reading, which may not always be reliable.
- Gestures and improvised communication.
- Relatives or accompanying persons acting as informal interpreters.
- The availability of professional sign-language interpreters.

These alternatives may introduce delays, misunderstandings, privacy concerns, and additional pressure on both patients and healthcare professionals.

The challenge is not simply the absence of communication. It is the difficulty of achieving clear, timely, and independent communication during healthcare interactions.

Interpreters see this daily, and see what it costs when the gap is filled by whoever happens to be nearby. That experience is not available in any dataset, and it is the reason this proposal is addressed to ASLIN first.

3 OUR PROPOSED SOLUTION

Unmute is developing an AI-assisted healthcare communication platform designed to facilitate communication between Deaf patients and hearing healthcare professionals.

The initial MVP will focus on American Sign Language (ASL), with a controlled set of healthcare-related expressions.

The intended capabilities include:

A. HEALTHCARE PROFESSIONAL TO DEAF PATIENT

- Spoken English input through a device microphone.
- Conversion of speech into text.
- Mapping of supported English expressions to corresponding ASL representations.
- Display of the corresponding sign-language video or avatar.

B. DEAF PATIENT TO HEALTHCARE PROFESSIONAL

- Camera-based capture of supported signing.
- Recognition of supported sign-language expressions.
- Conversion of recognised expressions into English text.
- Optional text-to-speech output.

C. COMMUNICATION SUPPORT

- Repetition and clarification options.
- Manual phrase selection where automatic recognition is unavailable.
- Clear indication when an expression is unsupported or cannot be confidently recognised.

The initial product will be limited to supported expressions and will be evaluated with Deaf users and healthcare professionals before any broader deployment.

Unmute is intended to support routine communication. It is not being presented as a replacement for qualified interpreters during emergencies, informed consent, complex clinical discussions, or other high-stakes interactions.

4 OBJECTIVES OF THE PROPOSED PARTNERSHIP

We seek to work with ASLIN to:

1. Review the initial clinical phrase list against the signs actually used in Nigerian healthcare settings.
2. Identify where American Sign Language and Nigerian Sign Language diverge, and where an ASL rendering would mislead rather than inform.
3. Obtain interpreter review of the proposed two-way workflow.
4. Agree how the product should behave when a sign is unsupported, ambiguous, or outside its vocabulary.
5. Understand the professional and ethical expectations ASLIN would place on a tool of this kind.
6. Agree a basis for crediting contributors and for describing ASLIN's involvement.

7 Where ASLIN considers it appropriate, facilitate introductions to healthcare institutions for a controlled evaluation.

5 PROPOSED AREAS OF COLLABORATION

We invite ASLIN to consider the following forms of involvement. Each is separable: ASLIN may take part in any one without the others.

A. LANGUAGE AND VOCABULARY REVIEW

- Review the healthcare expressions proposed for the MVP.
- Advise on signs used in Nigerian clinical practice, including regional and local variation.
- Identify expressions that should not be attempted without a qualified interpreter present.

B. INTERPRETER REVIEW

- Nominate one or more interpreters to review the proposed workflow.
- Advise on how the product should signal uncertainty and unsupported signs.
- Comment on where routine communication ends and high-stakes communication begins.

C. INTRODUCTIONS

- Where ASLIN considers it appropriate, introduce Unmute to healthcare institutions.
- Take part in introductory discussions if ASLIN wishes to.

D. POSITION AND ENDORSEMENT

ASLIN is under no obligation to endorse the product. No endorsement is requested or implied at this stage, and none will be described as existing until it is given in writing.

6 WHAT WE ARE REQUESTING FROM ASLIN

We are not requesting a financial contribution, an endorsement, or an exclusive arrangement.

We are seeking an initial conversation, and if ASLIN is willing, the following:

- A working session on the clinical phrase list, for which Unmute would meet reasonable interpreter time and costs.
- Written comment on the proposed workflow from one or more nominated interpreters.
- Agreement on how contributors are credited.
- Agreement, in advance and in writing, on how ASLIN's involvement is described publicly.

No partnership, endorsement, or agreement exists between Unmute and ASLIN until one is signed. This document is an invitation to talk, not a claim of association.

7 RESPONSIBILITIES OF UNMUTE

Unmute intends to:

- Develop and maintain the MVP.
- Fund working sessions, including reasonable interpreter time and costs.
- Clearly communicate supported capabilities and known limitations.
- Credit contributors as agreed.
- Not describe ASLIN as a partner, endorser, or adviser without written agreement.
- Provide a written summary of what was reviewed and what changed as a result.
- Publish the limitations of the product rather than only its capabilities.
- Work with ASLIN to define suitable next steps.

8 EXPECTED OUTCOMES

The proposed collaboration is expected to help:

- Produce a phrase list that reflects Nigerian practice rather than an imported one.
- Identify early where ASL fails Nigerian Deaf patients, before a hospital discovers it.
- Give interpreters a stated role in how the tool behaves, rather than only in what it gets wrong.
- Establish a defensible basis for a controlled clinical evaluation.
- Ensure the product's limits are described publicly and consistently.
- Contribute to more accessible healthcare communication.

9 PROPOSED NEXT STEPS

We propose the following engagement process:

1. ASLIN reviews this proposal and the proposed phrase list.
2. An introductory meeting is held at ASLIN's convenience.
3. A working session on the phrase list is scheduled and funded by Unmute.
4. Unmute provides a written summary of what was reviewed and what changed.
5. ASLIN decides whether it wishes to facilitate introductions to healthcare institutions.
6. Any further relationship is agreed in writing before it is described publicly.

10 CONCLUSION

Accessible communication is an important part of equitable healthcare delivery.

Unmute believes technology can help reduce some of the barriers Deaf patients encounter when communicating with healthcare professionals. However, building a useful solution requires collaboration with the institutions and people who experience these challenges directly.

We welcome the opportunity to work with ASLIN to evaluate this approach, improve the product through practical feedback, and explore a sustainable model for accessible healthcare communication.

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